



Clinical Case Quiz

A 60-year-old male noticed a tender bump on his leg, measuring about 1 – 2 cm in size. He’s not sure how long it has been there, but he noticed it a week or two ago. The patient sent the photograph found on the right. Based on the brief history and the photograph, the next best step is to:



- a. *Treat with doxycycline to cover for MRSA, as this is likely a cellulitis*
- b. *Send the patient to urgent care for incision and drainage*
- c. *Send a Telederm consult to his local area*
- d. *Submit a formal dermatology consult*
- e. *Either C or D*

Best answer: e. Send a Telederm consult to his local area or submit a formal dermatology consult

This skin lesion has a classic appearance for squamous cell skin cancer. These cancers can appear to be an erythematous and hyperkeratotic papule on top of the skin. Ulceration may be present. Definitive diagnosis is made by biopsy and histopathologic examination.

Dermatology input is necessary, and while a formal consult may be made, the patient’s care would be handled more efficiently via a Telederm consult to his local area.

Below are other examples of how squamous cell skin cancer can appear.



Note. From *Skin Cancer, Squamous Cell Carcinoma, Face* [Photograph], by Kelly Nelson, 2012, National Cancer Institute (<https://visualsonline.cancer.gov/details.cfm?imageid=9248>).



Note. From *Squamous Cell Skin Cancer* [Photograph], by Wellington Dermatology, n.d., Wellington Dermatology (<https://wellingtondermatology.nz/services/squamous-cell-skin-cancer/>).



Note. From *Skin Cancer, Squamous Cell Carcinoma, Leg* [Photograph], by Kelly Nelson, 2012, National Cancer Institute (<https://visualsonline.cancer.gov/details.cfm?imageid=9249>).

Making a Telederm Referral

The following steps will help make a referral to virtual dermatology easy:

1. Open a telephone encounter and enter in the dot phrase: .vmctelederm
 - This encounter should be in the home instance of the patient. Under “reason for call,” type, “Virtual Dermatology Visit.”
2. Make sure a clear photo of the skin lesion is in the media section of the chart, or can be inserted into your note, for consultation.
3. Follow the steps to fill out the questions in the dot phrase.
4. Route the telephone message to the appropriate pool indicated in this dot phrase.
5. Make sure you do not close the encounter. Keep it open. Leaving the encounter open will allow for proper processing.
6. Do not send a Dr Advice to dermatology. Do not input the dotphrase into your GCN or COD encounter note. Follow the above steps to submit Telederm consults.

Dermatology will review the message and then determine the next steps in helping provide further care for the member, whether it be recommendations for treatment or potential advisement for a follow up with dermatology. If a live consultation is needed, then the dermatology department will directly reach out to the member to schedule an appointment.

[See the attached FAQ for teledermatology](#) for more information.

References

Nelson, K. (2012). *Skin Cancer, Squamous Cell Carcinoma, Face* [Photograph]. National Cancer Institute. <https://visualsonline.cancer.gov/details.cfm?imageid=9248>

Nelson, K. (2012). *Skin Cancer, Squamous Cell Carcinoma, Leg* [Photograph]. National Cancer Institute. <https://visualsonline.cancer.gov/details.cfm?imageid=9249>

Saint John’s Cancer Institute. (n.d.) *What Does Squamous Cell Carcinoma Look Like* [Photograph]. Saint John’s Cancer Institute. <https://www.saintjohnscancer.org/melanoma/conditions/squamous-cell-carcinoma-of-the-skin/>

Wellington Dermatology. (n.d.) *Squamous Cell Skin Cancer* [Photograph]. Wellington Dermatology. <https://wellingtondermatology.nz/services/squamous-cell-skin-cancer/>